

48 mm

95 mm

28 mm

500 mm

SPACE FOR PHARMACODE

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For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

IMIQUIMOD CREAM USP 5% w/w

Imiqui

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CREAM

COMPOSITION

Each 0.25 gm single use sachet contains:

Imiquimod USP12.5mg (5.00% w/w)

Cream baseq.s.

Preservatives:

Methyl Paraben0.20% w/w

Propyl Paraben0.02% w/w

Benzyl Alcohol2.10% w/w

Product Description: White to off-white soft cream

Pharmacodynamics:

Pharmacodynamic properties

Pharmacotherapeutic group: Chemotherapeutics for topical use, antivirals:
ATC Code: D06BB10. Imiquimod is an immune response modifier. Saturable binding studies suggest a membrane receptor for imiquimod exists on responding immune cells. Imiquimod has no direct antiviral activity. In animal models imiquimod is effective against viral infections and acts as an anti-tumour agent principally by induction of alpha interferon and other cytokines.
The induction of alpha interferon and other cytokines following imiquimod cream application to genital wart tissue has also been demonstrated. Increases in systemic levels of alpha interferon and other cytokines following topical application of imiquimod were demonstrated in a pharmacokinetic study.

Pharmacokinetics:

External genital warts, superficial basal cell carcinoma and actinic keratosis:
Less than 0.9% of a topically applied single dose of radiolabelled imiquimod was absorbed through the skin of human subjects. The small amount of drug which was absorbed into the systemic circulation was promptly excreted by both urinary and fecal routes at a mean ratio of approximately 3 to 1.
No quantifiable levels (>5 ng/ml) of drug were detected in serum after single or multiple topical doses.
Systemic exposure (percutaneous penetration) was calculated from recovery of carbon-14 from [C] imiquimod in urine and faeces.
Minimal systemic absorption of imiquimod 5% cream across the skin of patients with actinic keratosis was observed with 3 times per week dosing for 16 weeks. The extent of percutaneous absorption did not change significantly between the first and last doses of this study. Peak serum drug concentrations at the end of week 16 were observed between 9 and 12 hours and were 0.1, 0.2, and 1.6 ng/mL for the applications to face (12.5 mg, 1 single-use sachet), scalp (25 mg, 2 sachets) and hands/arms (75 mg, 6 sachets), respectively. The application surface area was not controlled in the scalp and hands/ arms groups. Dose proportionality was not observed. An apparent half-life was calculated that was approximately 10 times greater than the 2 hour half-life seen following subcutaneous dosing in a previous study, suggesting prolonged retention of drug in the skin. Urinary recovery was less than 0.6% of the applied dose at week 16 in these patients.

Paediatric patients:

The pharmacokinetic properties of imiquimod following single and multiple topical application in paediatric patients with molluscum contagiosum (MC) have been investigated. The systemic exposure data demonstrated that the extent of absorption of imiquimod following topical application to the MC lesional skin of the paediatric patients aged 6-12 years was low and comparable to that observed in healthy adults and adults with actinic keratosis or superficial basal cell carcinoma. In younger patients aged 2-5 years absorption, based on C values, was higher compared to adults.

Indication:

Imiquimod cream is indicated for:
1. Treatment of external genital and perianal warts/condyloma accuminata in adults.
2. Treatment of clinically typical, nonhyperkeratotic, nonhypertrophic actinic keratoses on the face or scalp in immunocompetent adults.
3. Treatment of biopsy-confirmed, primary superficial basal cell carcinoma (sBCC) in immunocompetent adults, with a maximum tumor diameter of 2.0 cm, located on the trunk (excluding anogenital skin), neck, or extremities (excluding hands and feet), only when surgical methods are medically less appropriate and patient follow-up can be reasonably assured.

The histological diagnosis of superficial basal cell carcinoma should be established prior to treatment, since safety and efficacy of imiquimod cream have not been established for other types of basal cell carcinomas, including nodular and morpheaform (fibrosing or sclerosing) types.

Recommended Dose

Posology and Method of Administration
The application frequency for Imiquimod cream is different for each indication.
External Genital/Perianal Warts: Imiquimod cream is to be applied three times per week, prior to normal sleeping hours, and left on the skin for 6-10 hours. A thin layer is to be applied to the wart area and rubbed in until the cream is no longer visible.
Imiquimod cream 5% is packaged in single-use packets which contain sufficient cream to cover a wart area of up to 20cm²; use of excessive amounts of cream should be avoided. Hand washing before and after cream application is recommended. The application site is not to be occluded.
During the 6-10 hours treatment period, showering or bathing should be avoided. Following the treatment period, Imiquimod cream should be removed by washing the treated area with mild soap and water. Examples of 3 times per week application schedules are; Monday, Wednesday, Friday; or Tuesday, Thursday, Saturday application prior to sleeping hours. Imiquimod treatment should continue until there is total clearance of the genital/perianal warts or for maximum of 16 weeks.
Local skin reactions (erythema) at the treatment site are common. A rest period of several days may be taken if required by the patient's discomfort or severity of the local skin reaction. Treatment may resume once the reaction subsides. Non-occlusive dressings such as cotton gauze or cotton underwear may be used in the management of skin reactions.
Actinic Keratosis: Imiquimod cream should be applied 2 times per week for a full 16 weeks to a defined treatment area on the face or scalp (but not both concurrently). The treatment area is defined as one contiguous area of approximately 25 cm² (e.g., 5 cm x 5 cm) on the face (e.g. forehead or one cheek) or on the scalp. Examples of 2 times per week application schedules are Monday and Thursday, or Tuesday and Friday. Imiquimod cream should be applied to the entire treatment area and rubbed in until the cream is no longer visible. No more than one packet of Imiquimod cream should be applied to the contiguous treatment area at each application. Imiquimod cream should be applied prior to normal sleeping hours and left on the skin for approximately 8 hours, after which time the cream should be removed by washing the area with mild soap and water. The prescriber should demonstrate the proper application technique to maximize the benefit of Imiquimod cream therapy.
It is recommended that patients wash their hands before and after applying Imiquimod cream. Before applying the cream, the patient should wash the treatment area with mild soap and water and allow the area to dry thoroughly (at least 10 minutes). Contact with the eyes, lips and nostrils should be avoided.
Local skin reactions in the treatment area are common. [see Adverse Reactions] A rest period of several days may be taken if required by the patient's discomfort or severity of the local skin reaction. However, the treatment period should not be extended beyond 16 weeks due to missed doses or rest periods. Response to treatment cannot be adequately assessed until resolution of local skin reactions. Lesions that do not respond to treatment should be carefully re-evaluated and management reconsidered.
Patients should be prescribed no more than 36 sachets for the 16-week treatment period. Unused sachets should be discarded. Partially-used sachets should be discarded and not reused.
Superficial Basal Cell Carcinoma (sBCC): Imiquimod cream should be applied 5 times per week for a full 6 weeks to a biopsy-confirmed superficial basal cell carcinoma. An example of a 5 times per week application schedule is to apply Imiquimod cream, once per day, Monday through Friday. Imiquimod cream should be applied prior to normal sleeping hours and left on the skin for approximately 8 hours, after which time the cream should be removed by washing the area with mild soap and water. The prescriber should demonstrate the proper application technique to maximize the benefit of Imiquimod cream therapy. It is recommended that patients wash their hands before and after applying Imiquimod cream. The patient should wash the treatment area with mild soap and water before applying the cream, and allow the area to dry thoroughly.
The target tumor should have a maximum diameter of 2 cm and be located on the trunk (excluding anogenital skin), neck, or extremities (excluding hands and feet). The treatment area should include a 1 cm margin of skin around the tumor. Sufficient cream should be applied to cover the treatment area, including 1 centimeter of skin surrounding the tumor. Imiquimod cream should be rubbed into the treatment area until the cream is no longer visible.

Table 1. Amount of Imiquimod cream to Use for sBCC

Target Tumor Diameter	Size of Cream Droplet to be Used (diameter)	Approximate Amount of Imiquimod Cream to be Used
0.5 to < 1.0 cm	4 mm	10 mg
≥ 1.0 to < 1.5 cm	5 mm	25 mg
≥ 1.5 to 2.0 cm	7 mm	40 mg

Contact with the eyes, lips and nostrils should be avoided.
Local skin reactions in the treatment area are common. [see Adverse Reactions] A rest period of several days may be taken if required by the patient's discomfort or severity of the local skin reaction.

Early clinical clearance cannot be adequately assessed until resolution of local skin reactions (e.g. 12 weeks post-treatment). Local skin reactions or other findings (e.g. infection) may require that a patient be seen sooner than the post-treatment assessment for clinical clearance. If there is clinical evidence of persistent tumor at the post-treatment assessment for clinical clearance, a biopsy or other alternative intervention should be considered.
Lesions that do not respond to therapy should be carefully re-evaluated and management reconsidered; the safety and efficacy of a repeat course of Imiquimod cream treatment have not been established. If any suspicious lesion arises in the treatment area at any time after a determination of clinical clearance, the patient should seek a medical evaluation.
Patients should be prescribed no more than 36 sachets for the 6-week treatment period. Unused sachets should be discarded. Partially-used sachets should be discarded and not reused.

Route of Administration: TOPICAL

Contraindications: Hypersensitivity to the active substance or to any of the excipients.

Warnings and Precautions:

External genital warts, superficial basal cell carcinoma and actinic keratosis:
Avoid contact with the eyes, lips and nostrils.
Imiquimod has the potential to exacerbate inflammatory conditions of the skin. Imiquimod cream should be used with caution in patients with autoimmune conditions.
Consideration should be given to balancing the benefit of imiquimod treatment for these patients with the risk associated with a possible worsening of their autoimmune condition.
Imiquimod cream should be used with caution in organ transplant patients. Consideration should be given to balancing the benefit of imiquimod treatment for these patients with the risk associated with the possibility of organ rejection or graft-versus-host disease.
Imiquimod cream therapy is not recommended until the skin has healed after any previous drug or surgical treatment. Application to broken skin could result in increased systemic absorption of imiquimod leading to a greater risk of adverse events.
The use of an occlusive dressing is not recommended with imiquimod cream therapy.
The excipients Cetyl alcohol, stearyl alcohol, methyl para hydroxybenzoate, propyl para hydroxybenzoate may cause allergic reactions. Benzyl alcohol may cause mild local irritation.
Rarely, intense local inflammatory reactions including skin weeping or erosion

As this preparation contains benzyl alcohol, its use should be avoided in children under two years of age.
Not to be used in neonates.

can occur after only a few applications of imiquimod cream. Local inflammatory reactions may be accompanied, or even preceded, by flu-like systemic signs and symptoms including malaise, pyrexia, nausea, myalgias and rigors. An interruption of dosing should be considered.
Imiquimod should be used with caution in patients with reduced haematologic reserve.

External genital warts:

There is limited experience in the use of imiquimod cream in the treatment of men with foreskin- associated warts. The safety database in uncircumcised men treated with imiquimod cream three times weekly and carrying out a daily foreskin hygiene routine is less than 100 patients. Treatment in this patient population is therefore recommended only in men who are able or willing to follow the daily foreskin hygiene routine. Early signs of stricture may include local skin reactions (e.g. erosion, ulceration, oedema, induration), or increasing difficulty in retracting the foreskin. If these symptoms occur, the treatment should be stopped immediately. Based on current knowledge, treating urethral, intra-vaginal, cervical, rectal or intra-anal warts is not recommended. Imiquimod cream therapy should not be initiated in tissues where open sores or wounds exist until after the area has healed.

Local skin reactions such as erythema, erosion, excoriation, flaking and oedema are common. Other local reactions such as induration, ulceration, scabbing, and vesicles have also been reported. Should an intolerable skin reaction occur, the cream should be removed by washing the area with mild soap and water. Treatment with imiquimod cream can be resumed after the skin reaction has moderated.

The risk of severe local skin reactions may be increased when imiquimod is used at higher than recommended doses. However, in rare cases severe local reactions that have required treatment and/or caused temporary incapacitation have been observed in patients who have used Imiquimod according to the instructions. Where such reactions have occurred at the urethral meatus, some women have experienced difficulty in urinating, sometimes requiring emergency catheterisation and treatment of the affected area.
No clinical experience exists with imiquimod cream immediately following treatment with other cutaneously applied drugs for treatment of external genital or perianal warts. Imiquimod cream should be washed from the skin before sexual activity. Imiquimod cream may weaken condoms and diaphragms, therefore concurrent use with imiquimod cream is not recommended. Alternative forms of contraception should be considered.
In immunocompromised patients, repeat treatment with imiquimod cream is not recommended. While limited data have shown an increased rate of wart reduction in HIV positive patients, imiquimod cream has not been shown to be as effective in terms of wart clearance in this patient group.

Superficial basal cell carcinoma:

Imiquimod has not been evaluated for the treatment of basal cell carcinoma within 1 cm of the eyelids, nose, lips or hairline.
During therapy and until healed, affected skin is likely to appear noticeably different from normal skin. Local skin reactions are common but these reactions generally decrease in intensity during therapy or resolve after cessation of imiquimod cream therapy. There is an association between the complete clearance rate and the intensity of local skin reactions (e.g. erythema). These local skin reactions may be related to the stimulation of local immune response. If required by the patient's discomfort or the severity of the local skin reaction, a rest period of several days may be taken.
Treatment with imiquimod cream can be resumed after the skin reaction has moderated.

The clinical outcome of therapy can be determined after regeneration of the treated skin, approximately 12 weeks after the end of treatment.
No clinical experience exists with the use of imiquimod cream in immunocompromised patients.
No clinical experience exists in patients with recurrent and previously treated BCCs, therefore use for previously treated tumours is not recommended.
Large tumours (>7.25 cm) are less likely to respond to imiquimod therapy.
The skin surface area treated should be protected from solar exposure.

Actinic keratosis:

Lesions clinically atypical for AK or suspicious for malignancy should be biopsied to determine appropriate treatment.
Imiquimod has not been evaluated for the treatment of actinic keratoses on the eyelids, the inside of the nostrils or ears, or the lip area inside the vermilion border.
There are very limited data available on the use of imiquimod for the treatment of actinic keratoses in anatomical locations other than the face and scalp. The available data on actinic keratosis on the forearms and hands do not support efficacy in this indication and therefore such use is not recommended.
Imiquimod is not recommended for the treatment of AK lesions with marked hyperkeratosis or hypertrophy as seen in cutaneous horns.
During therapy and until healed, affected skin is likely to appear noticeably different from normal skin. Local skin reactions are common but these reactions generally decrease in intensity during therapy or resolve after cessation of imiquimod cream therapy. There is an association between the complete clearance rate and the intensity of local skin reactions (e.g. erythema). These local skin reactions may be related to the stimulation of local immune response. If required by the patient's discomfort or the intensity of the local skin reaction, a rest period of several days may be taken.
Treatment with imiquimod cream can be resumed after the skin reaction has moderated.
Each treatment period should not be extended beyond 4 weeks due to missed doses or rest periods. The clinical outcome of therapy can be determined after regeneration of the treated skin, approximately 4-8 weeks after the end of treatment.
No clinical experience exists with the use of imiquimod cream in immunocompromised patients.
The skin surface area treated should be protected from solar exposure.

Interactions with Other Medicaments

No interaction studies have been performed. This includes studies with immunosuppressive drugs. Interactions with systemic drugs would be limited by the minimal percutaneous absorption of imiquimod cream. Due to its immunostimulating properties, imiquimod cream should be used with caution in patients who are receiving immunosuppressive medication.

Pregnancy and Lactation

Pregnancy

For imiquimod no clinical data on exposed pregnancies are available. Animal studies do not indicate direct or indirect harmful effects with respect to pregnancy, embryonal/foetal development, parturition or postnatal development. Caution should be exercised when prescribing to pregnant women.

Breast-feeding

As no quantifiable levels (>5 ng/ml) of imiquimod are detected in the serum after single and multiple topical doses, no specific advice can be given on whether to use or not in lactating mothers

Side Effects:

Undesirable effects

a. General Description: **External genital warts:**
The most frequent adverse drug reactions judged to be probably or possibly related to imiquimod cream treatment (3 times a week dosing) were

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SAME SIZE ARTWORK
LEAFLET SIZE: 190 mm x 500 mm

application site reactions at the wart treatment site. The systemic adverse reactions considered at least possibly causally related to treatment with imiquimod were headache, influenza-like symptoms, and myalgia.

Superficial basal cell carcinoma:
The most frequent adverse events judged probably or possibly related to imiquimod cream (5 times per week dosing) are application site disorders. The systemic adverse reactions considered at least possibly causally related to treatment with imiquimod were back pain and influenza-like symptoms.

Actinic keratosis
The most frequent adverse event judged probably or possibly related to imiquimod cream (3 times per week dosing for up to 2 courses each of 4 weeks) was application site reactions. The systemic adverse reaction considered at least possibly causally related to treatment with imiquimod was myalgia.

b. Tabular Listing of adverse events:

Frequencies are defined as Very common, Common and Uncommon.

	External genital warts (3x/ wk, 16wks)	Superficial basal cell carcinoma (5x/wk, 6 wks)	Actinic keratosis (3x/wk, 4 or 8 wks)
Infections and infestations:			
Infection	Common	Common	Uncommon
Pustules		Common	Uncommon
Herpes simplex	Uncommon		
Genital candidiasis	Uncommon		
Vaginitis	Uncommon		
Bacterial infection	Uncommon		
Fungal infection	Uncommon		
Upper respiratory tract infection	Uncommon		
Vulvitis	Uncommon		
Rhinitis			Uncommon
Influenza			Uncommon
Blood and lymphatic system disorders:			
Lymphadenopathy	Uncommon	Common	Uncommon
Metabolism and nutrition disorders:			
Anorexia	Uncommon		Common
Psychiatric disorders:			
Insomnia	Uncommon		
Depression	Uncommon		Uncommon
Irritability		Uncommon	
Nervous system disorders:			
Headache	Common		Common
Paraesthesia	Uncommon		
Dizziness	Uncommon		
Migraine	Uncommon		
Somnolence	Uncommon		
Eye disorders			
Conjunctival irritation			Uncommon
Eyelid oedema			Uncommon
Ear and labyrinth disorders:			
Tinnitus	Uncommon		
Vascular disorders:			
Flushing	Uncommon		
Respiratory, thoracic and mediastinal disorders:			
Pharyngitis	Uncommon		
Rhinitis	Uncommon		
Nasal congestion			Uncommon
Pharyngo laryngeal pain			Uncommon
Gastrointestinal disorders:			
Nausea	Common	Uncommon	Common
Abdominal pain	Uncommon		
Diarrhoea	Uncommon		Uncommon
Vomiting	Uncommon		
Rectal disorder	Uncommon		
Rectal tenesmus	Uncommon		
Dry mouth		Uncommon	
Skin and subcutaneous tissue disorders:			
Pruritus	Uncommon		
Dermatitis	Uncommon	Uncommon	
Folliculitis	Uncommon		
Rash erythematous	Uncommon		
Eczema	Uncommon		
Rash	Uncommon		
Sweating increased	Uncommon		
Urticaria	Uncommon		
Actinic keratosis			Uncommon
Erythema			Uncommon
Face oedema			Uncommon
Skin ulcer			Uncommon
Musculoskeletal and connective tissue disorders:			
Myalgia	Common		Common
Arthralgia	Uncommon		Common
Back pain	Uncommon	Common	
Pain in extremity			Uncommon
Renal and urinary disorders:			
Dysuria	Uncommon		
Reproductive system and breast disorders:			
Genital pain male	Uncommon		
Penile disorder	Uncommon		
Dyspareunia	Uncommon		
Erectile dysfunction	Uncommon		
Uterovaginal prolapse	Uncommon		
Vaginal pain	Uncommon		
Vaginitis atrophic	Uncommon		
Vulval disorder	Uncommon		
General disorders and administration site conditions:			
Application site pruritus	Very common	Very common	Very common
Application site pain	Very common	Common	Common
Application site burning	Common	Common	Common
Application site irritation	Common	Common	Common
Application site erythema		Common	Common
Application site reaction			Common

Application site bleeding		Common	Uncommon
Application site papules		Common	Uncommon
Application site paraesthesia		Common	Uncommon
Application site rash		Common	
Fatigue	Common		Common
Pyrexia	Uncommon		Uncommon
Influenza-like illness	Uncommon	Uncommon	
Pain	Uncommon		
Asthenia	Uncommon		Uncommon
Malaise	Uncommon		
Rigors	Uncommon		Uncommon
Application site dermatitis			Uncommon
Application site discharge		Uncommon	Uncommon
Application site hyperaesthesia			Uncommon
Application site inflammation		Uncommon	
Application site oedema		Uncommon	Uncommon
Application site scabbing		Uncommon	Uncommon
Application site scar			Uncommon
Application site skin breakdown		Uncommon	
Application site swelling		Uncommon	Uncommon
Application site ulcer			Uncommon
Application site vesicles		Uncommon	Uncommon
Application site warmth			Uncommon
Lethargy		Uncommon	
Discomfort			Uncommon
Inflammation			Uncommon

c. Frequently occurring adverse events: **External genital warts:**
Local skin reactions including erythema, erosion, excoriation/flaking/scaling and oedema were common with imiquimod cream applied three times weekly. Local skin reactions, such as erythema, are probably an extension of the pharmacologic effects of imiquimod cream.

Superficial basal cell carcinoma:
Severe erythema, severe erosions and severe scabbing and crusting were very common with imiquimod cream applied 5 times weekly. Local skin reactions, such as erythema, are probably an extension of the pharmacologic effect of imiquimod cream.

Skin infections during treatment with imiquimod have been observed. While serious sequelae have not resulted, the possibility of infection in broken skin should always be considered.

Actinic keratosis
The most frequently occurring application site reactions with imiquimod cream 3 times weekly for 4 or 8 weeks were itching at the target site and burning at the target site. Severe erythema and severe scabbing and crusting were very common. Local skin reactions, such as erythema, are probably an extension of the pharmacologic effect of imiquimod cream.

Skin infections during treatment with imiquimod have been observed. While serious sequelae have not resulted, the possibility of infection in broken skin should always be considered.

d. Adverse events applicable to all indications:
The use of imiquimod for the treatment of actinic keratosis have detected alopecia at the treatment site or surrounding area.

Symptoms and Treatment of Overdose
When applied topically, systemic overdosage with imiquimod cream is unlikely due to minimal percutaneous absorption. Studies in rabbits reveal a dermal lethal dose of greater than 5 g/kg. Persistent dermal overdosing of imiquimod cream could result in severe local skin reactions.

Following accidental ingestion, nausea, emesis, headache, myalgia and fever could occur after a single dose of 200 mg imiquimod which corresponds to the content of approximately 16 sachets. The most clinically serious adverse event reported following multiple oral doses of ≥ 200 mg was hypotension which resolved following oral or intravenous fluid administration.

Effects on Ability to Drive and Use Machine
Imiquimod cream has no or negligible influence on the ability to drive and use machines.

Preclinical Safety Data
Non-clinical data revealed no special hazard for humans based on conventional studies of safety pharmacology, mutagenicity and teratogenicity.

ATC Code: D06BB10 IMIQUIMOD

Herbal Code: N/A

PHARMACEUTICAL PARTICULARS

List of excipients
The inactive ingredients are *Isostearic acid, Cetyl alcohol, Stearyl alcohol, White Petrolatum, Polysorbate 60, Sorbitan Monosterate, Methyl Paraben, Propyl Paraben, Glycerin, Xanthan gum, Benzyl alcohol and Purified water.*

Incompatibilities
Not applicable

Instructions for Use
Not Applicable

Shelf life
24 months

Storage Condition
Store below 30°C. Avoid freezing.

Pack Size
Pack of 3 sachets

Special precautions for disposal and other handling
No special requirements

MARKETING AUTHORISATION NUMBER(S):
N/A

DATE OF FIRST AUTHORISATION/RENEWAL OF THE AUTHORISATION:
N/A

DATE OF REVISION OF THE TEXT
30 October 2019

Product Registration Holder:
Glenmark Pharmaceuticals (Malaysia) Sdn Bhd (660397-W)
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59200 Kuala Lumpur, Malaysia

Manufactured by:
glenmark
Pharmaceuticals Ltd.
At: Plot No. S-7, Colvale Industrial Estate,
Colvale, Bardez, Goa - 403 513, India.
TM Trade Mark

PE00000 MY

500 mm

PHARMACODE:

ICONGRAPHICS CODE:

GLENMARK PHARMACEUTICALS LTD.				This is an Important Document. Please handle it Carefully. Initials & Remarks only in Space Provided below.				PANTONE SHADE		
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Product Code	:	PExxxxx			Version	:				
Item	:	LEAFLET			Location	:	GOA			
Reason For Changing of A/W:										
DEPARTMENT		CHECK LIST FOR APPROVAL				APPROVED BY:				
Packaging Development		Item code, Version, Consistency of Design, over print area, Pack Size, Carton size, label size, foil width, repeat length, specification & Layout								
Regulatory affair		Regulatory Text.								
Corporate QA		Entire Text Matter								
Site QA		Entire Text Matter								
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